جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ماجده احمد محمود محمد
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** ~~
رقم البطاقة : 28206120102543 **رقم التليفون :** 01207104446 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 12-06-1982 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ماجده احمد محمود محمد
National ID: 28206120102543

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



National ID · Front
[[DOCTYPE:ID_FRONT]]



National ID · Back
[[DOCTYPE:ID_BACK]]

Name: ماجدة احمد محمود محمد	
MR. : 442	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient underwent neuroimaging in January 2026, which demonstrated multiple ischemic foci on brain MRI. In February 2026, a chest CT revealed left lower lobar nodules and a hyper-fluid lesion (HFL) prompting recommendation for dynamic MRI evaluation. In April 2026, liver MRI showed a hyper-fluid lesion consistent with likely fatty infiltration and a uterine subserosal fibroid. Multidisciplinary discussion in February 2026 recommended initiation of aramidex combined with zoladex 10.8 mg. The current plan includes continuation of aramidex with supportive care for three months.

تحريرا في ,

التاريخ :	**18-09-2026**	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

MDT FORM 1/2/2025 G

mds 442

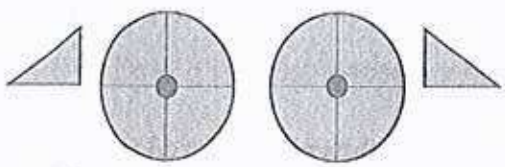
Name	ماجدة احمد محمود	MRN	٣٤٨٤١٢٢
		ID	٢٨٥٠٦١٠١٠٤٥٤
Age	٤٣ سنه	Phone	٠١٠٩١٩٥٢٥٠٠
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	—		

Clinical Data

BX:

ER:	PR:	HER 2:	Ki67:
bil: 8/8	8/8	+2 not amplified	

R L



Sonomammography:

2022: up Front:
Rt mastectomy → T1aN1
Mets Workup: 1+ BCS + SLNB → T2 N0 NM

2023: AC x4 + TXT x4
PORTH

Recommendations: tamoxifen

11/2025: mri brain → multiple ischemic foci
Hormonal profile: (R)

MDT OUTCOME & PLAN

Pdt on TAM ~ 1.5y — Note the MRI Brain → ischemic changes
+ shyt to AI / Zoladex
for Arimidex / Zoladex after E2 level

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY
INTERNATIONAL
Egypte

Name : Magda Ahmed Mahmoud	**Date : 09-02-2026 11:26**
Branch : Portable	**Medical No. : GRI-17115**
Ref.Physician Prof : CAP+C	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of managed right cancer breast,, no available clinical data or previous examination.

I- CHEST:

- Right portacath is noted.
- Right MRM showed clear operative bed with no evidence of enhanced focal lesion or fluid collection.
- Prominent left axillary lymph nodes, the largest measures about 11mm .
- Left lower lobar tiny nodule measures about 5mm.
- Middle lobar subpleural fine reticulations with bilateral basal atelectatic bands are noted. Likely post-radiation sequel
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Average cardiac size,

II- ABDOMEN AND PELVIS:





- The liver is enlarged in size showing homogenous fatty parenchymal attenuation. The left hepatic lobe segment IVb showed fairly defined hypodense area in non-contrast study measures about 2.3x1.9 cm another suspected isodense area is seen in segment IVa measures about 2 cm (red circle) **for further dynamic MRI.**
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : Magda Ahmed Mahmoud	**Date : 09-02-2026 11:26**
Branch : Portable	**Medical No. : GRI-17115**
Ref.Physician Prof : CAP+C	**Ins.Company :**

or back pressure changes. Right lower parapelvic cyst with marginal calcification is seen measuring about 1.7x1.6 cm. right lower cortical calcification measures about 1.7 cm

- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No ascites.
- Bilateral enlarged inguinal lymph nodes, the largest is seen on left side measures about 1.5 cm
- **The examined skeleton** no worrisome lytic or sclerotic lesions.

Opinion:

- **Left lower lobar tiny nodule.**
- **Fatty hepatomegaly with left hepatic focal lesion and suspected right hepatic lesion for further dynamic MRI.**
- **Right lower para-pelvic cyst.**

Dr.Norhan Ramzy,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

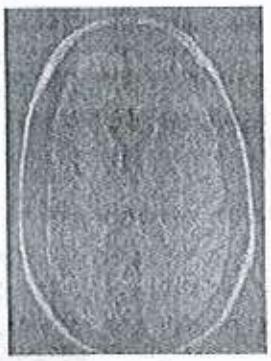
GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient Name: Magda Ahmed Mahmoud Mohamed	Date : 27/11/2025
Exam : Item 1	Age : 43 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 17115

OPEN MRI EXAMINATION OF THE BRAIN

TECHNIQUE:

Axial T1, T2, FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs
Post contrast T1 WIs



FINDINGS:

- Normal size and configuration of the ventricular system.
- No midline shift or deformity.
- No evidence of cerebral, cerebellar or brain stem areas of abnormal signal apart from few bright white matter foci are known as unidentified bright objects [UBO], they are incidentally found in many patients, their exact etiology is unknown, they are suspected to be of old ischemic nature, headache related, dilated Virchow Robbins spaces or otherwise.
- Normal cortical sulci and extra axial CSF spaces.
- No evidence of extra axial collections.
- No cerebello pontine angle masses.
- No evidence of posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

Few bright white matter foci, they are incidentally found in many patients, their exact etiology is unknown, they are suspected to be of old ischemic nature, headache related, dilated Virchow Robbins spaces or otherwise, for clinical correlation and follow up by closed not open MRI to exclude possibility of demyelinating plaques.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

M.R.I
[[DOCTYPE:MRI]]

Patient Name	: الأستاذه / ماجدة احمد محمود	Request No.	: 13004816
Patient No.	: 28206120102543	Request Date	: 31/10/2022
Path No.	: 3266/22	Verification Date	: 27/11/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Microscopic examination:
Right MRM:
- Histological type: Tiny focus of invasive duct carcinoma, NST
- Grade (differentiation+ nuclear pleomorphism + mitosis): II (3+2+1)
- Angioinvasion: Positive
- Ductal carcinoma in situ: Extensive (cribriform, solid, micropapillary, comedo), high grade
- Lymph node involvement: Positive metastatic deposits in 2/20 axillary lymph nodes
- Resection Margins: Free

Left WLE and SLNs biopsy:
- Histological type: Invasive mixed lobular and duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II (3+2+1)
- Angioinvasion: Negative
- Ductal carcinoma in situ: 30% (cribriform, solid), intermediate grade
- Lymph node involvement: Free all SLNs 0/6
- Resection Margins: Free with fibrocystic changes.
- Positive perineural invasion

Diagnosis:
•Right MRM specimen:
- Tiny focus of invasive duct carcinoma, NST, grade II
- With Extensive high grade DCIS component
- Free nipple and areola
- Free deep margin
- Positive metastatic deposits in 2/20 axillary lymph nodes
- pT1a pN1 pMx.

•Left WLE ans SLNs biopsy specimens:
- Invasive mixed lobular and duct carcinoma, grade II
- Associated 30% DCIS, intermediate grade

Verified by :
walaa ghoniem

Room No. (4) - First Floor Building No. 89 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ماجدة احمد محمود	Request No.	: 13004816
Patient No.	: 28206120102543	Request Date	: 31/10/2022
Path No.	: 3266/22	Verification Date	: 27/11/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Free margins and extra-margin
- Free SLNs (0/6)
- pT2 pN0 pMx

Prof.Dr. Manal El Mahdy
Dr.Walaa Ghonaim

Verified by :
walaa ghoniem

Room No. (6) - First Floor Building No. 287- Nile Corniche - Misr El Qadema- Cairo.
Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ماجدة احمد محمود	Request No.	: 13006467
Patient No.	: 28206120102543	Request Date	: 20/08/2022
Path No.	: 2411/22	Verification Date	: 19/09/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

CLINICAL DATA:
Right& Left breast masses.

NATURE OF SPECIMEN:
1- Left breast mass, Tru cut biopsy, collected at 20/8/2022, 11:00 am.
2- Right breast mass, Tru cut biopsy, collected at 20/8/2022, 11:15 am.
GROSS EXAMINATION:
1- Left breast: Three grayish white tissue cores up to 1.2 cm, totally submitted.
2- Right breast: Multiple grayish white tissue cores up to 1 cm, totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

MICROSCOPIC EXAMINATION:
1- Left breast: * Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism +mitosis): (3+2+2) Grade II.
* DCIS: Focal.
* Angioinvasion: Positive.
ER: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
Her-2: Moderate complete membrane reaction in >10% of tumor cells (score 2+)
Ki 67: 10 % nuclear reaction
Average Her2 copy No : 1.65
Her2/CEP17 ratio : 1.1
SISH : Not amplified.
2- Right breast: * Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism +mitosis): (3+2+2) Grade II.
* DCIS: Focal.

Verified by :
DR.Israa Elgendy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ماجدة احمد محمود	Request No.	: 13006467
Patient No.	: 28206120102543	Request Date	: 20/08/2022
Path No.	: 2411/22	Verification Date	: 19/09/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

* Angioinvasion: No.
ER: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
Her-2: Moderate complete membrane reaction in >10% of tumor cells (score 2+)
Ki 67: 10 % nuclear reaction
Average Her2 copy No : 2.3
Her2/CEP17 ratio : 1.2
SISH : Not amplified.

DIAGNOSIS:
Right breast masses, Tru cut biopsy:
Invasive duct carcinoma, Grade II.
ER : Positive (8/8)
PR : Positive (8/8)
HER2 : Equivocal (score 2+).
Ki67: 10%.
SISH : Not amplified.
Left breast masses, Tru cut biopsy:
Invasive duct carcinoma, Grade II.
ER : Positive (8/8).
PR : Positive (8/8).
HER2 : Equivocal (score 2+).
Ki67: 10%.
SISH : Not amplified.

Prof. Dr. Gamal Abd Elaty.
Dr. Esraa Elgendy.

Verified by :
DR.Israa Elgendy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - الوترنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ماجدة احمد محمود		
Patient No.	: 28206120102543	Request No.	: 13006467
Path No.	: 2411/22	Request Date	: 20/08/2022
Sex	: Female	Verification Date	: 06/09/2022
Age	: 40 Y	Financial Category	: اورام دار السلام - مباشره
Referreing Doc.	:	Department	: Out Patient
		IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA:
Right& Left breast masses.

NATURE OF SPECIMEN:
1- Left breast mass, Tru cut biopsy, collected at 20/8/2022, 11:00 am.
2- Right breast mass, Tru cut biopsy, collected at 20/8/2022, 11:15 am.

GROSS EXAMINATION:
1- Left breast: Three grayish white tissue cores up to 1.2 cm, totally submitted.
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MICROSCOPIC EXAMINATION:
1- Left breast: * Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism +mitosis): (3+2+2) Grade II.
* DCIS: Focal.
* Angioinvasion: Positive.
ER: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
Her-2: Moderate complete membrane reaction in >10% of tumor cells (score 2+)
Ki 67: 10 % nuclear reaction

2- Right breast: * Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism +mitosis): (3+2+2) Grade II.
* DCIS: Focal.
* Angioinvasion: No.
ER: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)

Verified by :
Dr.Marwa mahmoud

Room No. (4) - First Floor-Building No. 937- Nile Corniche - Misr El Qadema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / ماجدة احمد محمود	Request No.	13006467
Patient No.	28206120102543	Request Date	20/08/2022
Path No.	2411/22	Verification Date	06/09/2022
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	40 Y	Department	Out Patient
Referreing Doc.		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

PR: Strong nuclear reaction in 60% of tumor cells (Allred score 8/8)
Her-2: Moderate complete membrane reaction in >10% of tumor cells (score 2+)
Ki 67: 10 % nuclear reaction

DIAGNOSIS:
Right breast masses, Tru cut biopsy:
Invasive duct carcinoma, Grade II.
ER : Positive (8/8)
PR : Positive (8/8)
HER2 : Equivocal (score 2+). Case for SISH
Ki67: 10%.
Left breast masses, Tru cut biopsy:
Invasive duct carcinoma, Grade II.
ER : Positive (8/8)
PR : Positive (8/8)
HER2 : Equivocal (score 2+). Case for SISH
Ki67: 10%

Prof. Dr. Gamal Abd Elaty.
Dr. Esraa Elgendy.

Verified by :
Dr.Marwa mahmoud

غرفة رقم (4) – الدور الأول – مبنى رقم (387) – كورنيش النيل – مدينة نصر القاهرة – مصر
Room No. (4) - First Floor-Building No. 387- Nile Corniche - Nasr El Qahera- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: *MAGDA Ahmed*	***Date of birth:*** *12/06/1982*
Arabic Name: ماجده احمد	***Patient Gender:*** *F*
Patient RIS ID: *334450* ***Patient HIS ID:*** *123016695*	***Exam Date:*** *16/08/2023*
Written by: Dr. Hoda Fathy	**Approved by:** Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (15/08/2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- No current definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Advanced active arthritic changes are noted over the right sterno-clavicular as well as both shoulder and both knee joints.
- Both kidneys are visualized with moderate to marked tracer stasis, for clinico-laboratory and/ or sonographic assessment.

Conclusion:

- No current definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Mayadah Ashraf

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Magda Ahmed Mahmoud Mohamed	Date : 17/08/2022
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 1634	Pat. ID: 17115

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Bilateral breast mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Heterogenous dense fibro glandular density (ACR C)
- Right UOQ segmental asymmetry with underlying suspicious grouped microcalcifications, overall area measuring about 124 x67 x117mm.
- Left UOQ suspicious clustered microcalcifications.
- Left LOQ spiculated dense mass lesion.

Complementary ultrasound :

- Right UOQ altered parenchyma with multiple ill-defined hypoechoic solid mass lesion seen at:
  1. Right 12 o'clock measuring about 5 x4mm.
  2. Right 1 o'clock measuring about 15 x11mm.
  3. Right 2 o'clock measuring about 7.5 x9.5mm.
  4. Right retro areolar measuring about 8 x7mm and 10 x9.5mm.
  5. Right 6 o'clock measuring about 7x7mm.
- Left 3 o'clock ill defined hypoechoic solid mass lesion measuring about 51 x23mm with highly internal vascularity seen within. Away from skin about 1.2mm and from nipple about 70mm.
- Left 10 o'clock ill defined hypoechoic solid mass lesion measuring about 3 x2mm.

- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts
- Multiple bilateral prominent axillary LNs (less than 3 nodes) with diffuse cortical thickening reaching to 5mmm at right side , left pathological about 4.7mm with effaced fatty hilum.
- Free supra and infra clavicular lymph nodes.

Opinion:

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name: Magda Ahmed mahmoud

Date: Tuesday, October 4, 2022

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Bilateral pathologically proven carcinoma

TECHNIQUE:

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Parenchymal enhancement(BPE).Minimal asymmetrical ACR C

LEFT BREAST:

- **Low energy (Non contrast) images** shows LOQ speculated obscured mass with grouped pleomorphic calcifications.
- **Recombined (Post contrast) images show** shows an LOQ mass oval speculated markedly heterogeneously enhancing measuring 26mm SSx36.3mm AP dimensions about 7CMFN
- **Complementary US confirmed the CESM findings.non specific axillary LN by US.**

RIGHT BREAST:

- **Low energy (Non contrast) images** shows UOQ grouped segmental malignant coarse heterogeneous and pleomorphic calcifications and small speculated masses and grouped calcifications in the LIQ.
- **Recombined (Post contrast) images** show UOQ segmental clumped NME measuring 104 mm SSx85 mmCC x 55 mm AP ending 29 mm from the nipple., no pathological uptake. innumerable (at least 8)small speculated enhancing masses are seen.
- **Complementary US confirmed the CESM finding with pathological level 1 two axillary LNs.**
- Normal skin / nipple / areolar complex.

IMPRESSION :

LEFT BREAST: unifocal mass as fore detailed (BIRADS 6).

RIGHT BREAST: Multicentric malignant disease as fore detailed (BIRADS 6)

Best regards,

Dr.Nivine Chalabi,MD.EBBI

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Presidential Program for Women's health

Name: Mgada ahmad
Date: 15-10-2022

Target axillary Ultrasound

Right Axilla

- Multiple enlarged pathological axillary LN at level 1 , with effaced hilum and cortical thickness reaching 5mm

Left Axilla

- Multiple enlarged LN with preserved hilum and cortical thickness not exceed 3mm

Dr. shaden Mahmoud

Mammogram
[[DOCTYPE:MAMMOGRAM]]

جمهورية مصر العربية

بطاقة تحقيق الشخصية



ماجده

احمد محمود محمد

١٣ محمد هلال- عزبة كامل صدقى البخريه

المنصرة - القاهره

٢٨٢٠٦١٢٠١٠٢٥٤٣

LB6997524

٢٨٢٠٦١٢٠١٠٢٥٤٣ ٢٠٢٥/٠٣

زوجة منزل

انثى مسلمة متزوجة

وليد محمود عبدالحليم سيد

١ البطاقة سارية حتى ٢٠٣٢/٠٣/١٤.

DMS

هرمونى

اراضيد كس + زوجد كس